

[Venous thrombosis, unrecognized complication of DRESS].

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DRESS (Drug Reaction with Eosinophilia and Systemic Symptoms) is a rare syndrome triggered by an immunological reaction to certain drugs and which may be life-threatening as a result of the onset of severe organ involvement. It is characterised by a long period from the time of drug therapy to the onset of actual signs. Herein, we report the case of 42-year-old female patient who developed DRESS one month after beginning allopurinol treatment. A 42-year-old woman was hospitalised for febrile exanthema with facial oedema, polyadenopathy, mononucleosis syndrome, major hypereosinophilia and hepatic cytolysis. A diagnosis was made of DRESS with a RegiSCAR score of 5. The implicated drug was allopurinol, which had been initiated one month earlier. HHV-6 IgM serology was positive. Two days after the start of systemic corticosteroids, the patient developed thrombosis of the internal jugular vein. Other than major hypereosinophilia, no other factors favouring thrombosis were detected. A favourable outcome was achieved under effective anticoagulants and corticosteroids. They have been rare reports of venous thrombosis during DRESS. Hypereosinophilia can be involved in the onset of this condition. Prophylaxis with systemic anticoagulants may be necessary in DRESS involving major hypereosinophilia.

DRESS syndrome: Part II. Management and therapeutics.

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The appropriate management of the drug reaction with eosinophilia and systemic symptoms (DRESS) syndrome is paramount because it is associated with significant morbidity and mortality. This syndrome shares clinical features with other dermatologic conditions, including other severe cutaneous drug reactions, requiring the clinician to carefully examine the proposed criteria to make the appropriate diagnosis. Once the diagnosis of DRESS syndrome has been established, the next step in management is immediate cessation of the causative medication(s). In cases in which the culprit drug is not obvious, clinicians must use their clinical judgment to select which medication to discontinue. They may also utilize patch or lymphocyte transformation tests to aid in identification when appropriate. Topical corticosteroids can be used for symptomatic relief, but systemic steroid therapy is generally required. Other immunosuppressants have also been employed in treatment and show promise in future therapy. Patients with DRESS syndrome should be managed in an intensive care or burn unit for appropriate care and infection control. In addition, appropriate specialists should be consulted based on the affected organ systems. Most patients recover completely after drug withdrawal and appropriate therapy. However, some patients with DRESS syndrome suffer from chronic complications and approximately 10% die, primarily from visceral organ compromise. Controlled clinical trials investigating the most appropriate therapies and their risks, particularly intravenous corticosteroids, are lacking, and would be invaluable in determining the optimal future treatment regimen for DRESS syndrome.

Drug reaction with eosinophilia and systemic symptoms (DRESS) in children.

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Drug Reaction with Eosinophilia and Systemic Symptoms (DRESS) is a severe reaction to drugs. Incidence of DRESS in children is not well known and the mortality rate seems to be lower than 10%.

Anticonvulsants are the main drugs involved both in adults and in children. The treatment of choice is intravenous immunoglobulins and corticosteroids used in synergy. Today there are not controlled clinical trials regarding DRESS treatment in children. Anyway, the prompt withdrawn of the offending drug is of paramount importance for a better prognosis. DRESS sequels may occur, consequently, follow-up visits are required at least until the first year after the reaction.

A case of drug reaction with eosinophilia and systemic symptoms.

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Drug reaction with eosinophilia and systemic symptoms (DRESS) is characterized by fever, skin rash, hematological abnormalities, and systemic involvement such as hepatitis. DRESS usually presents 2-6 weeks after drug initiation. DRESS should be suspected on clinical grounds in the setting of the introduction of new drug therapy and is most commonly described after the introduction of aromatic anticonvulsants, allopurinol, or antiretroviral therapies. We describe here a case of DRESS due to phenytoin exposure with complete resolution on drug discontinuation. Our patient developed DRESS with a skin rash, lymphadenopathy, and markedly abnormal liver enzymes, 4 weeks after drug initiation following drainage of a brain abscess. He was initially diagnosed as having a recurrence of the abscess or sepsis of another origin. It is important to recognise the possibility of DRESS in this setting, as a good outcome depends on the immediate withdrawal of the offending drug. A mortality rate of up to 10% has been described in unrecognised cases.

Cyclophosphamide therapy for corticoresistant drug reaction with eosinophilia and systemic symptoms (DRESS) syndrome in a patient with severe kidney and eye involvement and Epstein-Barr virus reactivation.

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DRESS (drug reaction with eosinophilia and systemic symptoms) syndrome is a severe adverse drug reaction with significant mortality, characterized by erythroderma, fever, lymphadenopathy, and visceral involvement. We report a case of multivisceral DRESS syndrome with posterior multifocal placoid pigment epitheliopathy and acute tubulointerstitial nephritis responsible for dialysis-dependent acute kidney failure in the context of reactivation of Epstein-Barr virus infection. Because of resistance of the skin and kidney manifestations to prolonged corticosteroid therapy, a 6-month course of oral cyclophosphamide resulted in complete recovery of all symptoms. To our knowledge, this is the first case showing the efficacy of cyclophosphamide in severe DRESS syndrome.

Relationship Between Clozapine-Induced Inflammation and Eosinophilia: A Retrospective Cohort Study.

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Eosinophilia has not been highlighted in clozapine-induced adverse inflammatory events, as it is often

asymptomatic and self-limiting, while drug reaction with eosinophilia and systemic symptoms (DRESS) syndrome occurs rarely. This study aimed to reveal the temporal relationships between eosinophilia and other inflammatory events during clozapine initiation. The temporal relationships between eosinophilia and other inflammatory events were evaluated among 241 patients with schizophrenia treated with clozapine for the first time at 7 hospitals. Risk factors for eosinophilia were investigated among preceding inflammatory events and other clinical characteristics. Furthermore, patients with eosinophilia were stratified by the severity of adverse inflammatory events and their clinical characteristics were compared. Of the 54 patients who experienced inflammatory adverse events, 27 (50%) developed eosinophilia. In all but 1 patient, clinical symptoms of inflammatory adverse events preceded eosinophilia. In contrast, of the 187 patients without inflammatory events, 21 (11%) developed eosinophilia. Multivariate analysis revealed that more severe preceding inflammatory adverse events were associated with a greater risk of eosinophilia. The median time to the first detection of eosinophilia and peak eosinophil count occurred significantly earlier in patients with severe adverse events than in asymptomatic patients. In most cases, eosinophilia developed after the onset of inflammatory symptoms. Preceding inflammation was associated with the development of clozapine-induced eosinophilia. Eosinophilia may not be suitable as an early detection marker of severe inflammatory adverse effects. These findings enhanced our understanding of the involvement of eosinophilia in clozapine-induced inflammatory events.

Drug Reaction With Eosinophilia and Systemic Symptoms Secondary to Trimethoprim/Sulfamethoxazole: A Case Report.

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Drug reaction with eosinophilia and systemic symptoms (DRESS syndrome) is a rare and potentially life-threatening condition. Symptoms typically manifest two to eight weeks after exposure to an offending agent, such as anticonvulsants and antibiotics. Clinical features include fever, morbilliform rash, eosinophilia, lymphadenopathy, and, in severe cases, multiorgan dysfunction, including interstitial nephritis, hepatitis, and pneumonitis. This case report discusses a 55-year-old female who developed DRESS syndrome while being treated for a urinary tract infection with trimethoprim/sulfamethoxazole (TMP/SMX). Presenting with fever, dysuria, flank pain, and a widespread non-pruritic maculopapular rash, her lab results were remarkable for eosinophilia, while the urinalysis revealed mild hematuria. Computed tomography imaging ruled out nephrolithiasis and acute pyelonephritis, leading to the diagnosis of DRESS syndrome. Management focused on discontinuing TMP/SMX and initiating systemic glucocorticoids. The patient responded well to treatment and was discharged on hospital day two with prescriptions for topical and oral steroids, famotidine, and diphenhydramine. The patient was provided with follow-up instructions and return precautions. Drug reaction with eosinophilia and systemic symptoms poses unique diagnostic challenges due to its similarity to other cutaneous reactions and the delay between drug exposure and symptom onset. Early recognition and intervention are vital for preventing severe complications. Given its potential for multiorgan dysfunction and poor patient outcomes, healthcare providers must be vigilant when evaluating patients presenting with fever and rash. Comprehensive history taking and accurate reconciliation of active and recent medications are necessary to make the diagnosis. Immediate discontinuation of the offending agent is essential, while supportive care and topical or systemic glucocorticoids remain the treatment standards.

DRESS Syndrome That Resembles Graft-Versus-Host Disease after Chemotherapy in a Pediatric Patient: A Case Report.

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Drug reaction with eosinophilia and systemic symptoms (DRESS) syndrome is a potentially life-threatening, drug-induced adverse reaction characterized by skin eruptions, lymphadenopathy, fever, and a broad range of other bodily manifestations. The spectrum of histopathologic and clinical presentations is wide; therefore, DRESS syndrome can mimic other diseases. We present a case of a 4-year-old male patient who started chemotherapy with vincristine, cytarabine, and etoposide. The first clinical signs were fever, hemodynamic instability, and maculopapular erythema. Biopsies of skin lesions were taken, and hyperkeratosis, focal parakeratosis, acanthosis with slight spongiosis, and intraepithelial dyskeratotic cells were observed. There was a perivascular lymphoid infiltrate with abundant eosinophils in the dermis, and eosinophil permeations to the acrosyringium and epithelium were found. DRESS syndrome is a drug-induced reaction that shares histopathological findings in skin biopsies with those seen in graft-versus-host disease. Although the histological findings are non-pathognomonic, they were characteristic enough to be of importance in the differential diagnosis.

Atypical Presentation of Drug Reaction With Eosinophilia and Systemic Symptoms (DRESS) Syndrome: When Gastrointestinal Symptoms Obscure the Diagnosis.

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The drug reaction with eosinophilia and systemic symptoms (DRESS) syndrome is a severe cutaneous adverse reaction. Due to its unfamiliarity, non-specific diagnostic criteria, and delayed onset, this condition is frequently overlooked, which can sometimes result in life-threatening consequences. DRESS typically manifests as an extensive mucocutaneous rash with multi-organ involvement. This report aims to emphasize the varied presentation of the syndrome. Our patient was initially presented with acute onset vomiting, abdominal pain, fever for a couple of days with a minor skin rash. At first, she was treated for acute viral gastritis. However, on her second presentation within a week, she had a more extensive skin rash. Upon detailed history, it was found that this was linked to the initiation of sulfasalazine for ulcerative colitis. RegiSCAR (Registry of Severe Cutaneous Adverse Reactions) scoring suggested it as a definite case of DRESS. The primary manifestation of gastrointestinal symptoms led to a delayed diagnosis. Still, it is important to consider the possibility of drug hypersensitivity when there are skin changes and blood abnormalities present.

Drug Reaction with Eosinophilia and Systemic Symptoms (DReSS)/Drug-Induced Hypersensitivity Syndrome (DiHS)-Readdressing the DReSS.

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Drug reaction with eosinophilia and systemic symptoms (DReSS), also known as drug-induced hypersensitivity syndrome (DiHS), is a severe, systemic, T cell mediated drug reaction with combinations of cutaneous, hematologic, and internal organ involvement. Pathogenesis of DReSS is multi-factorial, involving drug-exposure, genetic predisposition through specific human leukocyte antigen (HLA) alleles and metabolism defects, viral reactivation, and immune dysregulation. Clinical features of this condition

are delayed, stepwise, and heterogenous, making this syndrome challenging to recognize and diagnose. Two sets of validated diagnostic criteria exist that can be employed to diagnose DReSS/DiHS. Methods to improve early recognition of DReSS and predict disease severity has been a recent area of research focus. In vitro and in vivo tests can be employed to confirm the diagnosis and help identify culprit drugs. The mainstay treatment of DReSS is prompt withdrawal of the culprit drug, supportive treatment, and immunosuppression depending on the severity of disease. We present a comprehensive review on the most recent research and literature on DReSS, with emphasis on pathogenesis, clinical features, diagnosis, confirmatory testing modalities, and treatment. Additionally, this summary aims to highlight the differing viewpoints on this severe disease and broaden our perspective on the condition known as DReSS.
